which unilaterally ended the state's commitment to supply water to Haryana. Even as the Supreme Court declared this legislation invalid in 2016, Badal in his last tenure denotified 4,627 acres of the 5,376 acres of land acquired for the canal in Rupnagar, Mohali, Patiala and Fatehgarh Sahib districts and returned it to the farmers for free in 2017.

Now, the Supreme Court wants the Centre to carry out a survey of this land. The Opposition leaders in Punjab maintain that Mann's law officers should have convinced the three-judge bench to review the water scenario in the state instead of opening a window for a survey that could very well lead to an adverse ruling for the state in days to come.

In Punjab, data from the water resources department shows that 109 of the 128 blocks are in the 'dark zone', that is, they have depleted groundwater levels. In Haryana, on the other hand, officials claim that groundwater in places like Gurugram district has fallen from a depth of 6.6 metres in 1976 to up to 30 metres now. BJP-ruled Haryana intends to use the additional water it could get from SYL to irrigate farms in the arid, rain-dependent southern parts of the state. Mann maintains, though, that Punjab will not share "even a single drop" with it. As the Centre prepares its status report, which it has to submit in the Supreme Court in January next year, the hostilities—both internal and external—are only likely to escalate in Punjab. ■



Illustration by SIDDHANT JUMDE

spread, with a very small proportion diagnosed at the early localised stage.

The CerviSCAN project, started by a research group of the RCC and engineers from the Centre for Development of Advanced Computing (C-DAC) in 2014, was funded by the Union ministry of electronics and information technology, the Department of Health Research and the Indian Council for Medical Research. The RCC and C-DAC completed the project in 2018. The device, developed under the Centre's Make in India initiative, has by now been validated by cancer specialists in four other institutes besides Thiruvananthapuram: the Dr Bhubaneswar Borooah Cancer Institute in Assam, the Agartala RCC and the New Delhi-based All India Institute of Medical Sciences and National Institute of Cancer Prevention and Research. The CerviSCAN technology will also be used for screening in Kerala's Wayanad district, a hilly region with a huge tribal population.

The high burden of cervical cancer in India and Southeast Asia has been largely attributed to poor living standards, a high prevalence of the Human Papilloma Virus (HPV) among women aged above 30, as well as inadequate screening. According to oncologists, almost all cervical cancers are linked to the high-risk HPV, which is transmitted sexually. "Cervical cancer occurs at an early age among women and strikes during their reproductive phase. The incidence rises in the 30–34 age band and peaks at 55–65 years, with a median age of 38 years (age 21–67 years). The only way to protect women is to vaccinate them against HPV at an early age," says Dr Nair.

In the absence of early vaccination, however, timely screening is the only hope for a large number of women. In developed countries, conventional cytology screening programmes have been shown to lead to a marked decline in the incidence of cervical cancer. Robust testing is the first step toward tackling India's cervical cancer burden. And CerviSCAN promises to be a much-needed leap forward on that front. ■